February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy, National Coordinator for Health Information Technology
Department of Health and Human Services
Attn: HHS Health Sector AI RFI
Mary E. Switzer Building
7033A, 330 C Street SW, Washington, DC 20201
Submitted Electronically at www.regulations.gov

Re: **[HHS Health Sector AI RFI]**
Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Dr. Keane:

On behalf of the over 60,000 members of the American Society of Anesthesiologists (ASA), I am pleased to offer comments to the Assistant Secretary for Technology Policy (ASTP) and the Department of Health and Human Services (HHS) on the *Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care* Request for Information (RFI). As a medical specialty society with a long track record of prioritizing patient safety, ASA appreciates the opportunity to provide feedback that will inform regulatory action by ASTP and HHS to expedite and improve the deployment of artificial intelligence (AI) solutions in clinical settings.

Patient safety, quality of care, and physician workflows can all be supported by the efficient implementation of AI technology. Sufficiently incorporating each of these crucial pillars of patient care into AI tools can only be achieved with communication between frontline clinical staff such as anesthesiologists, policymakers, and AI developers and vendors.

ASA's comments to ASTP's RFI questions reflect our desire to help remove significant barriers to AI tool adoption in anesthesia settings. Anesthesiologists and AI developers must work to establish trust in the emerging technology, address ease-of-use concerns that can worsen rather than mitigate workflow burdens, and overcome the lack of financial incentives for both frontline health care workers and facility decision-makers.

The approach by HHS and ASTP to streamline health IT certification requirements and broader regulations around AI clearly demonstrates the federal government's "try-first" message to AI developers. ASA supports the agencies' aim to accelerate the development and implementation of innovative AI solutions in health care settings, ensuring that valuable tools reach our members and their patients sooner. However, we also believe this approach could be compatible with regulatory action that supports greater transparency, real-world testing, and improved incentive structures – each of these objectives would help adapt the development and deployment of AI tools to the implementation barriers faced by anesthesiologists and their facilities. Clear guidance from HHS and ASTP on issues like liability, privacy, and security could also mitigate burdens on small and mid-sized facilities and anesthesia practices that may not have the resources to adequately build a governance structure for AI use on their own.

Please see our responses to each of the RFI questions below.

1. ***What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?***

AI solutions for anesthesia care are best positioned for widespread adoption and successful use if developers partner with anesthesiologists, nurses, and perioperative teams through all stages of development, rather than just engaging clinical stakeholders for validation. For example, in the perioperative setting, predictive models for surgical case duration often fail because the models are designed without significant input from anesthesiologists. Anesthesiologists understand non-linear factors like surgeon experience, case complexity evolution, and team dynamics – all features that can help in AI design and implementation. Models trained with an overreliance on historical data miss these critical variables.

AI developers and ASTP should also focus on building trust among physicians and other medical staff. AI developers should conduct assessments on the causes of any reluctance or delays in implementing AI in health care. For instance, some physicians may be concerned about an AI application using a “black box model” that fails to translate into clinical interpretability. This occurs when a tool provides a recommendation without sufficient detail about why the model produced that recommendation. In other cases, an AI model may lack standardized validation metrics specific to clinical workflows. Physicians may also wish to have more information or data about a tool’s performance data. Last, clinical stakeholders may be concerned about relying too much on AI recommendations.

ASA encourages ASTP to incentivize "explainable AI by default" standards. Clinical AI tools must provide interpretable reasoning alongside predictions, similar to how radiologists provide differential diagnoses rather than offering a final read. ASTP may wish to consider incorporating such standards as a requirement in its Certified Electronic Health Record Technology (CEHRT) and clinical decision support standards.

Clinician ease-of-use considerations also present a barrier to clinical adoption of AI tools. Tools that disrupt established workflows or add steps to those workflows, at least in the early stages of adoption, are more likely to face resistance from potential users. Similarly, clinicians often face increased workflow burdens after an AI tool is initially integrated into existing EHR systems. A clear example of this is reflected in anesthesiologists’ extensive use of patient monitors. AI tools for anesthesia care that use alerts on monitoring devices can contribute to alarm fatigue if they have a poorly calibrated prediction threshold.

Separately, the use of clinically valuable AI tools often lacks a sustainable payment structure even when there are demonstrated improvements in care processes. **HHS should coordinate with the Centers for Medicare and Medicaid Services (CMS) and private payers on how the effective use of AI tools with demonstrated value in patient safety and quality of care can be incentivized through payment levers.**

2. ***What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.***

HHS should develop a programmatic structure for the evaluation of the quality, safety, and efficiency associated with the use of AI tools. Such a structure would assist hospitals and health systems who are deploying AI tools, anticipating that cost savings or improved efficiency may accrue, but whose benefits are mostly theoretical at this point. HHS should dedicate funding for studies to assess the deployment of AI in healthcare, including automation bias, explainability, comprehensiveness, hallucination and its mitigation, errors of omission, model drift, and model bias.

3. ***For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?***

Current liability frameworks provide little clarity on whether the AI developer, the anesthesiologist, or the health care facility bears responsibility for legal, privacy, and security issues that may result from the use of clinical AI tools. ASA believes that physicians should use their clinical judgement when assessing or using tools that lack comprehensive rationale for their recommendations. At the same time, developers may be less incentivized to invest in addressing gaps if all liability for those clinical gaps or errors rests with the facility and its clinical staff.

HHS should convene an expert interdisciplinary task force to develop a firm, risk-based policy framework around governance for liability, privacy, and security in the use of clinical AI tools. This task force should coordinate with relevant HHS subagencies, such as the Food and Drug Administration (FDA), and clarify developer and deployer responsibilities, disclosure expectations, accountability, and post-deployment monitoring requirements.

4. ***For non-medical devices, what are the most promising AI evaluation methods (pre- and postdeployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?***

ASA appreciated the FDA's emphasis on AI lifecycle, transparency, and bias mitigation in its January 2025 guidance, *Artificial Intelligence-Enabled Device Software Functions: Lifecycle Management and Marketing Submission Recommendations*. **HHS should extend these principles to non-device AI through programmatic funding and standardized evaluation frameworks**, as they would apply to high-impact clinical AI tools that do not fit into FDA's definition of a device.

We also support future HHS funding of grant programs for testing AI validation methodologies in real-world clinical environments. These programs should allow for interdisciplinary expertise and comprehensive evaluation across all phases of the translational spectrum. For example, continuous, real-world performance monitoring is significantly more valuable than one-time pre-deployment validation. Yet anesthesiologists would benefit from broader studies on AI validation methodologies. AI tools may predict hypotension accurately in aggregate but, in some cases, they may fail for specific patient subgroups, such as patients with autonomic dysfunction.

5. ***How can HHS best support private sector activities (e.g., accreditation, certification, industry driven testing, and credentialing) to promote innovative and effective AI use in clinical care?***

HHS can effectively promote innovative and safe AI use by developing clear guidance for accreditation, certification, testing, and credentialing, while enabling trusted private-sector organizations to operationalize these standards. By articulating risk-based expectations for AI governance, supporting independent evaluation infrastructure, encouraging clinician competency frameworks, and aligning federal programs with recognized certifications, HHS can reduce uncertainty, improve accountability, and accelerate responsible AI adoption without imposing rigid or premature regulation.

6. ***Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?***

AI tools have seen significant success with applications that augment image- and signal-based diagnostics with clear workflows, measurable performance, and explicit payment pathways (e.g., AI-ECG; AI coronary plaque analysis). AI-powered administrative automation tools have also supported workflow burden reduction and improved efficiency.

AI tools that have failed to meaningfully improve clinical care have often suffered from opaque models, weak validation, unclear or suboptimal clinical integration pathways, and/or the imposition of high false-positive burden (e.g., seizure detection with high false alarms). Relatedly, the high volume of alarms for some AI tools has contributed to alarm fatigue for anesthesiologists.

For anesthesiologists, the greatest perioperative potential for AI tools lies in real-time intraoperative co-pilot systems integrating multi-monitor data for context-aware guidance, personalized anesthesia planning predicting individual medication responses, predictive operating room efficiency optimization, pain management personalization, and just-in-time clinical education surfacing protocols during rare crisis scenarios.

7. ***Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?***

Within facilities and health systems, the adoption of AI tools for clinical care is most strongly influenced by financial and executive leadership, particularly chief financial officers whose decisions hinge on perceived or actual efficiency gains and cost reductions. While these executives are not responsible for identifying or implementing AI tools, they may stall the adoption of these tools if integration costs are underfunded or if regulatory or payer requirements are unclear. Facility stakeholders and anesthesiologists wish to have greater certainty around potential payment mechanisms or incentives to offset integration and governance costs. Addressing these barriers through CMS policy would have a positive impact on AI adoption at scale.

On the other hand, frontline physician champions often determine the success of AI tool implementation. Primary hurdles to achieving local buy-in for AI tools include fragmented governance across departments, IT resource constraints and training capacity, liability uncertainty, lack of standardized validation frameworks, siloed data infrastructure, and misaligned incentives in which efficiency gains do not reward physicians and other relevant staff for driving adoption.

8. ***Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.***

Interoperability is a critically necessary but insufficient condition for the improvement of public health and individual patients through IT and AI. There is no international standard for the interoperable sharing of comprehensive anesthesia records. This is partly the result of how the anesthesia record is structured, containing a wide breadth of information that includes medication administration, vital signs, ventilator data, procedure and perioperative evaluation notes, and more. Perioperative medication data, procedure complexity indicators, and loss of longitudinal patient data across care settings are details that have limited interoperable deployment. These nuances are critical for practicing anesthesiologists. Further, certain data, including high-frequency physiologic waveforms are often locked in proprietary systems or not easily extractable.

The pathway to interoperability for anesthesia records would benefit from FHIR extensions for anesthesia-specific data, standardized performance metrics, open validation datasets, and model cards documenting training data and limitations. **HHS should specifically deploy resources for the completion of the HL7 data model for anesthesiology**. This one standard would enable third-party innovation, cross-institutional validation, and broad data access while accelerating safe AI development.

To fuel research, HHS should support standardization that enables access to high-fidelity, streaming physiological data (e.g., waveform data, capnography, plethysmography) rather than just static discrete data points entered into an EHR. Enhanced interoperability would encourage "closed-loop" systems in which anesthesia machines, infusion pumps, and physiological monitors would seamlessly communicate with one another. This would create a market for third-party AI developers to build "clinical driver assistance" tools, with algorithms that sit on top of device streams to predict patient conditions like hypotension or depth of anesthesia without being locked into a single manufacturer's hardware.

9. ***What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?***

Physician interactions can be critical in comforting and supporting patients throughout the perioperative process, and AI tools should not limit those interactions or weaken personalized care. Anesthesiologists often observe that patients seek personalized risk assessment, reduced wait times, better pain management, enhanced safety monitoring, and improved communication. Patients also want to be assured that the physician caring for them remains in control of their care, adhering to the "co-pilot, not autopilot" principle. In the same vein, any AI-powered recommendation should be accompanied by a transparent and comprehensive rationale so that care decisions can adequately be explained to patients. Anesthesiologists are privy to these patient needs and concerns and should be empowered to use AI in a way that can mitigate potential biases and improve patient care and safety.

10. ***Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?***
    a. ***Are there published findings about the impact of adopted AI tools and their use clinical care?***
    b. ***How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?***

HHS should prioritize implementation science and studying actual AI usage patterns based on real-world evidence, automation bias, cognitive load (including a focus on alarm fatigue), and workflow integration rather than just technical validation. Longitudinal studies on model degradation would also be helpful in addressing drift.

HHS may also wish to consider cost-benefit analyses across hospitals and health systems, including those with fewer financial resources. While high upfront costs may remain an obstacle to purchasing decisions, calculating the cost of inaction and, on the flip side, improvements in efficiency through the use of AI tools, can address this barrier. Specifically, anesthesiologists and their facilities may benefit from learning more about the role AI tools can play in recovery optimization, improved pain management, and the prevention or mitigation of postoperative nausea and vomiting. Each of these outcomes contributes to patient length of stay and clinical resources devoted to the patient.

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Thank you for your consideration of our comments. ASA welcomes the opportunity to speak with ASTP and HHS further about our feedback. Please contact Matthew Goldan, Senior Quality Operations Associate (m.goldan@asahq.org), for questions or further information.

Sincerely,

Patrick Giam, MD, FASA
President
American Society of Anesthesiologists